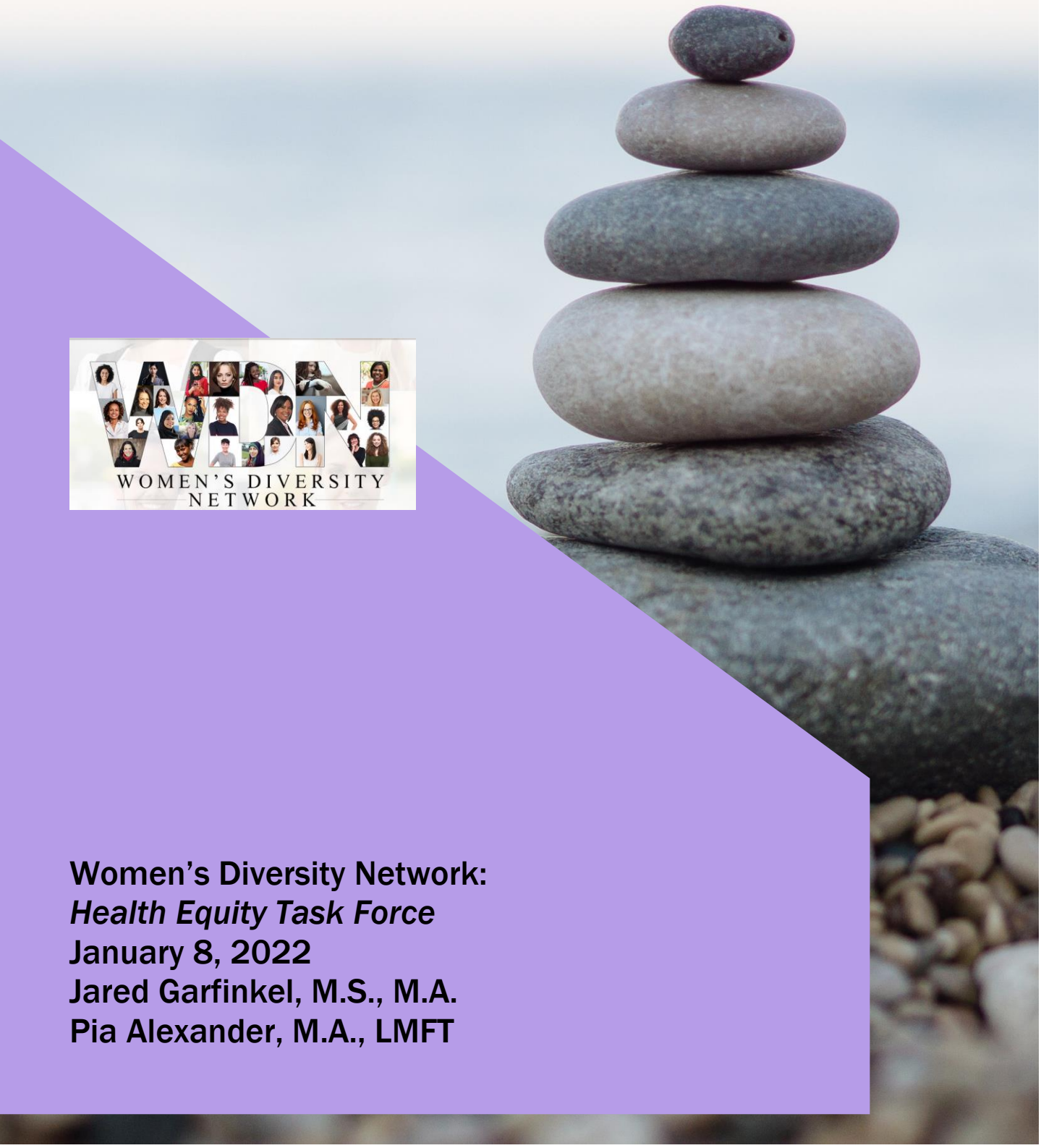
# Nassau and Suffolk County Listening Sessions Report




Women's Diversity Network:
*Health Equity Task Force*
January 8, 2022
Jared Garfinkel, M.S., M.A.
Pia Alexander, M.A., LMFT

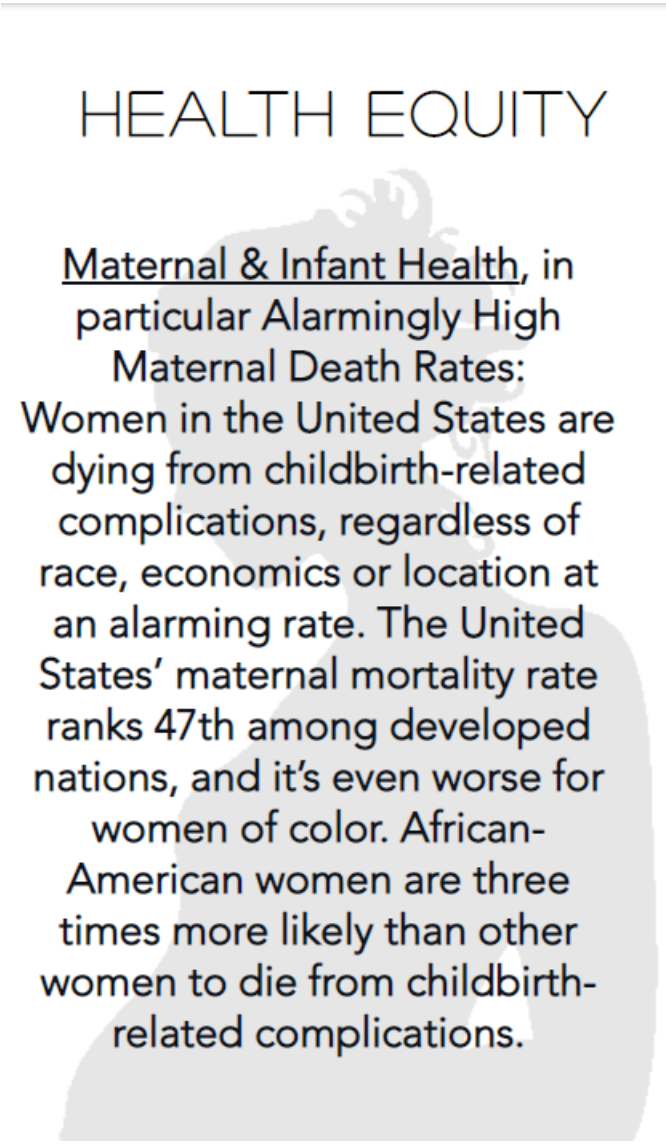
# Introduction

## The Women's Diversity Network (WDN)

The Women's Diversity Network (WDN) is a NY-based non-profit organization comprised of over 100 members and working with more than 50 partner organizations throughout New York State. It provides programming, resources, and training along three organized advocacy areas: Racial Bias & Systemic Racism, Gender Violence, and Health Equity.

The WDN Health Equity Task Force (HETF) was organized in 2018 to create and sustain consistent accountability across health systems and healthcare providers, and to improve health outcomes for community members, particularly Black women and those identifying as part of a vulnerable population across Long Island.

The HETF has had significant success in increasing awareness of health disparities and Health Equity (HE) in maternal, infant, and child health and is an active advocacy platform for policy change on Long Island.




https://womensdiversitynetwork.org




Figure 1. Word cloud produced from Nassau County Listening Session participant responses https://www.freewordcloudgenerator.com/generatewordcloud

# Background

Public health campaigns throughout history have raised consciousness about the importance of hygiene, clean water, clean air, and an environment free from toxins. Public health can support our communities. Unfortunately, the history of public health research shows that it can also uphold racist policies in disregard for human subjects.

While the 20th century saw many of the great accomplishments of public health programs come to fruition, it was also home to great failures of practitioners to treat people ethically and humanely. Even in the realm of reproductive health, one need only search the trials of birth control to find that scientists willfully flouted basic medical ethics to treat women in unfair ways in the development of "the pill". Nevertheless, the opportunity for public health programs that mitigate health risks is still viable.

Counter to recent gains in many areas of public health, the maternal mortality rate has risen since the 1990s. And, Black maternal mortality is higher than that of other racial and ethnic groups. Black non-Hispanic women were overrepresented in a 2014 pregnancy-related death cohort. While 51.5% of pregnancy related deaths were from Black non-Hispanic women in a 2014 report, only 15% of total pregnancies were from

Black non-Hispanic women. Furthermore, nearly one-third of pregnancy related deaths were preventable.

This urgent issue remains relevant today on Long Island. The Suffolk County Legislature approved a maternal mortality task force to combat racism in maternal health outcomes in late 2020. WDN as a community organization is represented on the newly formed Suffolk County Maternal Mortality and Morbidity Task Force, and the feedback from these virtual forums will be shared in support of the goals of the aforementioned taskforce and any future committees seeking to better understand and improve equitable access to care, remove barriers to care, and to elevate the standards of care that women receive from health care systems and direct providers.

As Black women in New York State are three times more likely than white women to die in childbirth than their white counterparts (NYS Taskforce of Maternal Mortality and Disparate Racial Outcomes, 2019), regardless of their educational or economic status, WDN identified an initial goal of organizing virtual listening sessions in Nassau County and Suffolk County Long Island in order to identify issues in two geographic areas that were not specifically covered in the NYS Department of Health Commissioner's Community Listening Sessions, which were held prior to the COVID 19 pandemic.

*New York State Maternal Mortality Review Report: A Comprehensive Review of the 2014 Cohort*. New York State Department of Health, 2014, https://www.health.ny.gov/community/adults/women/docs/maternal_mortality_review_2014.pdf.

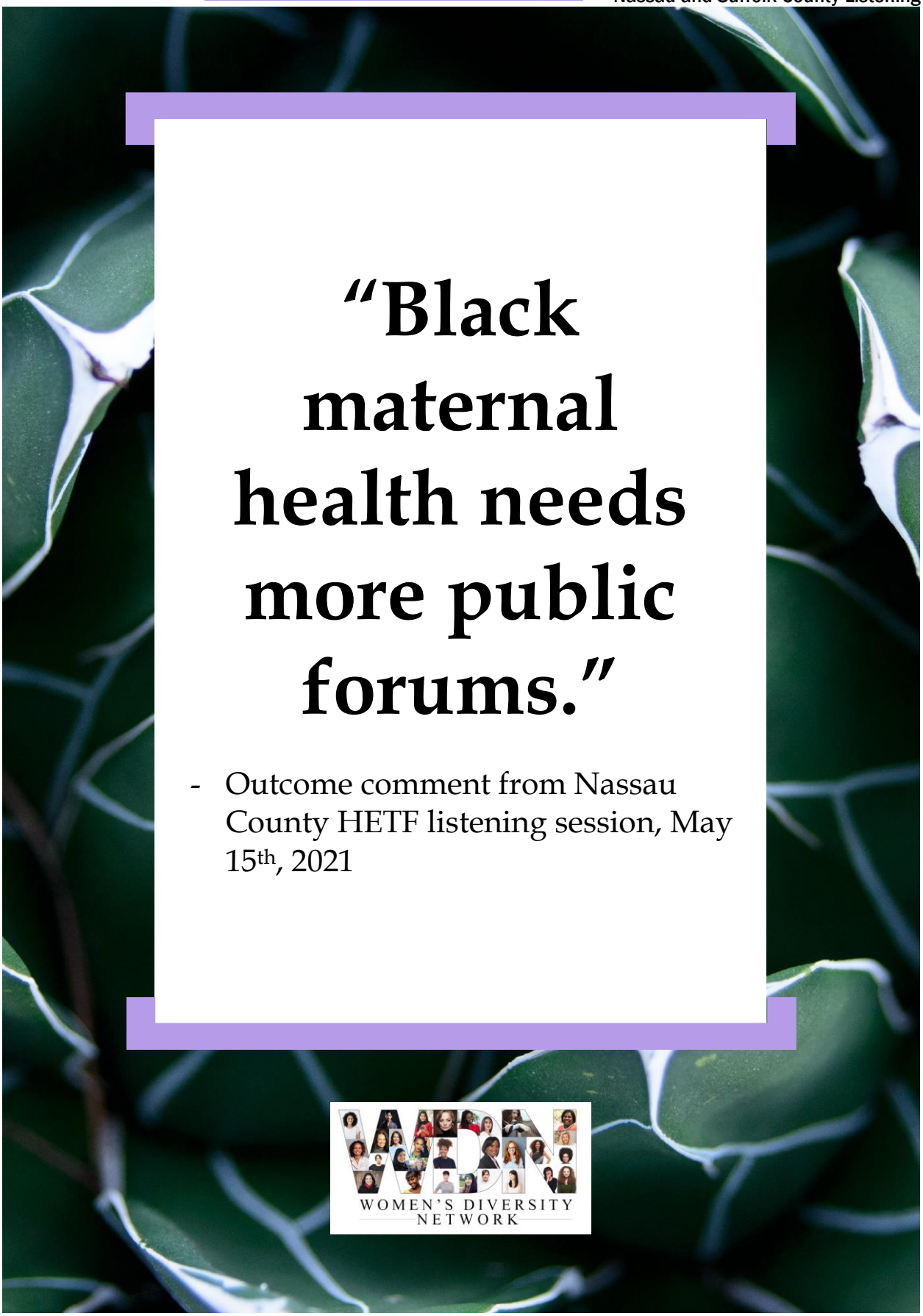
Blidner, Rachelle. "Lawmakers Approve Maternal Mortality Task Force." *Newsday*, Newsday, 16 Dec. 2020, https://www.newsday.com/long-island/suffolk/suffolk-legislature-maternal-morality-1.50093429.

Hunter-Grant, Cheryl, and Mary Rosser. "New York State Expert Panel on Postpartum Care." *Taskforce on Maternal Mortality and Disparate Racial Outcomes*, New York State Department of Health, Jan. 2021, https://www.health.ny.gov/community/adults/women/task_force_maternal_mortality/docs/2021-01_expert_panel_on_postpartum_care_final_report.pdf.

Rabin, Roni Caryn. "Huge Racial Disparities Found in Deaths Linked to Pregnancy." *The New York Times*, The New York Times, 7 May 2019, https://www.nytimes.com/2019/05/07/health/pregnancy-deaths-.html#:~:text=African-American%2C%20Native%20American%20and%20Alaska%20Native%20women%20are,Times%20By%20Roni%20Caryn%20Rabin%20May%207%2C%202019.

Villarosa, Linda. "Why America's Black Mothers and Babies Are in a Life-or-Death Crisis." *The New York Times*, The New York Times, 11 Apr. 2018, https://www.nytimes.com/2018/04/11/magazine/black-mothers-babies-death-maternal-mortality.html.

Zucker, Howard, et al. "Maternal Mortality Report." *New York State Taskforce on Maternal Mortality and Disparate Racial Outcomes*, New York State Department of Health, Mar. 2019, https://health.ny.gov/community/adults/women/task_force_maternal_mortality/docs/maternal_mortality_report.pdf.



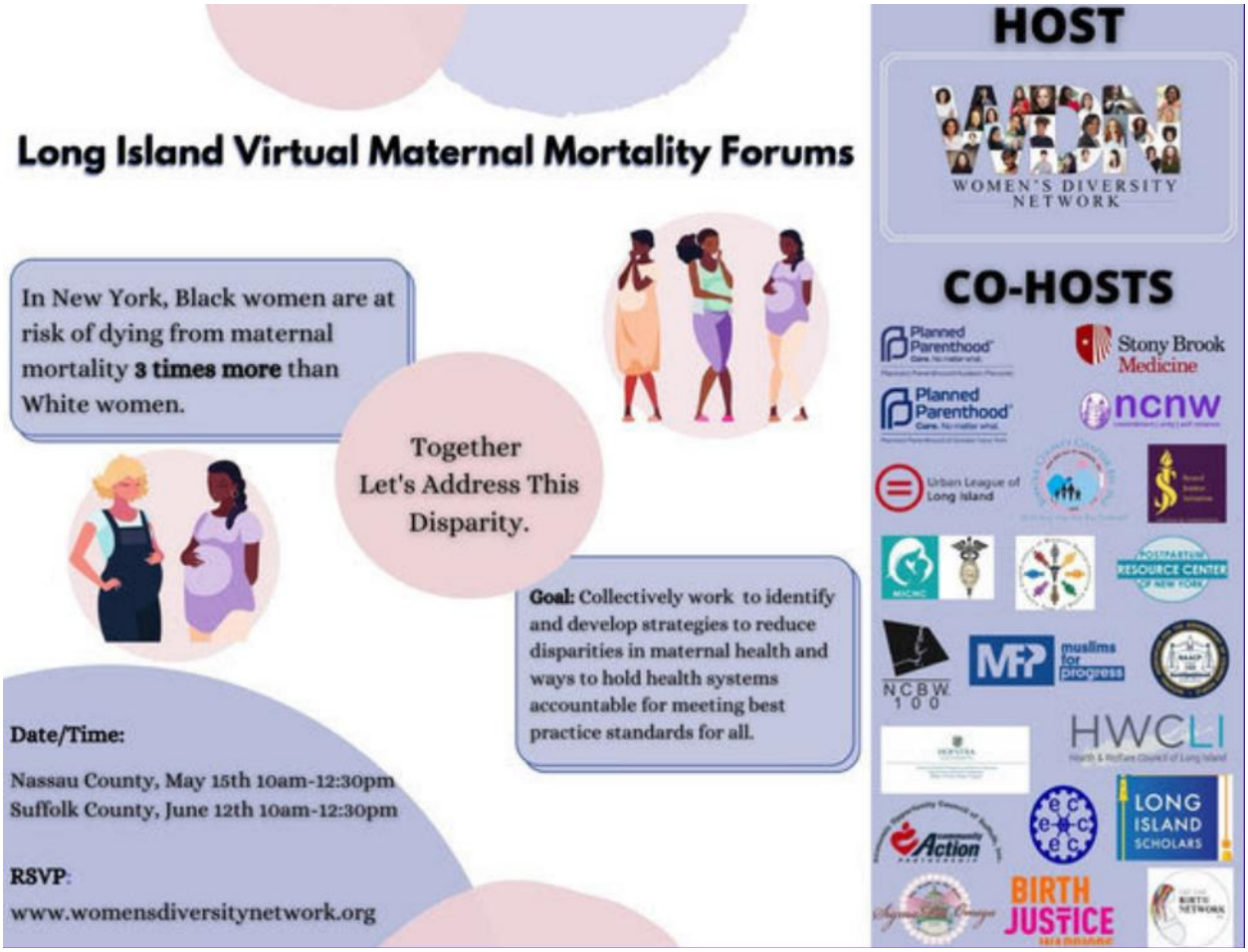
# "Black maternal health needs more public forums."

- Outcome comment from Nassau County HETF listening session, May 15th, 2021

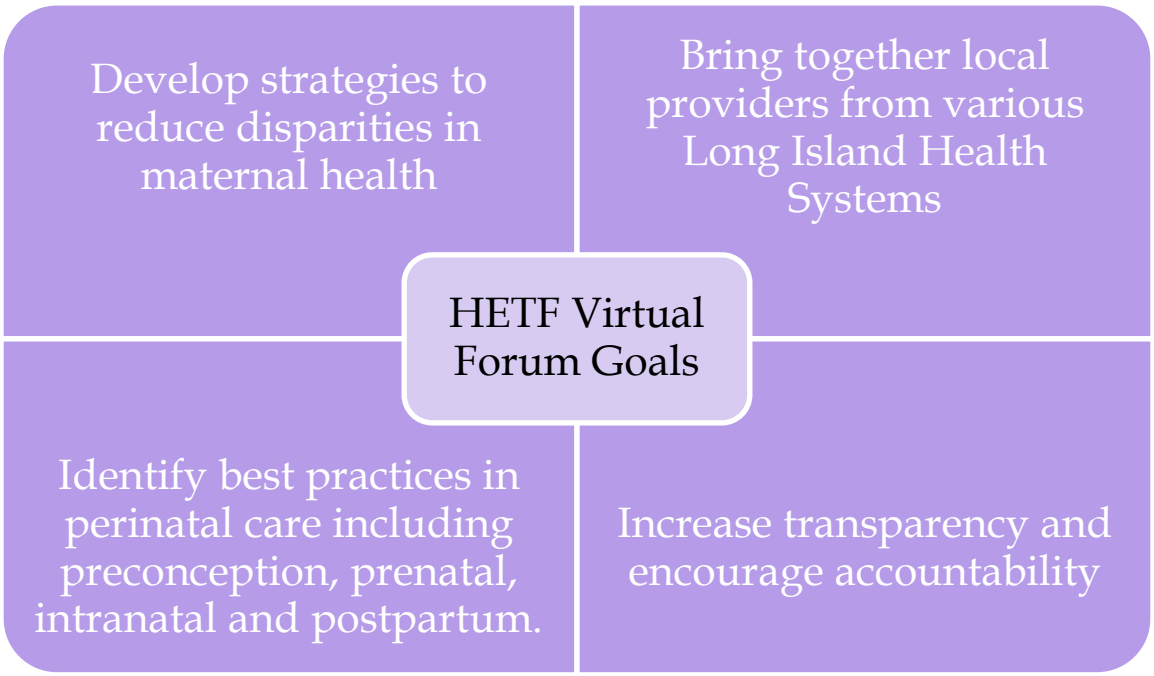
WOMEN'S DIVERSITY NETWORK

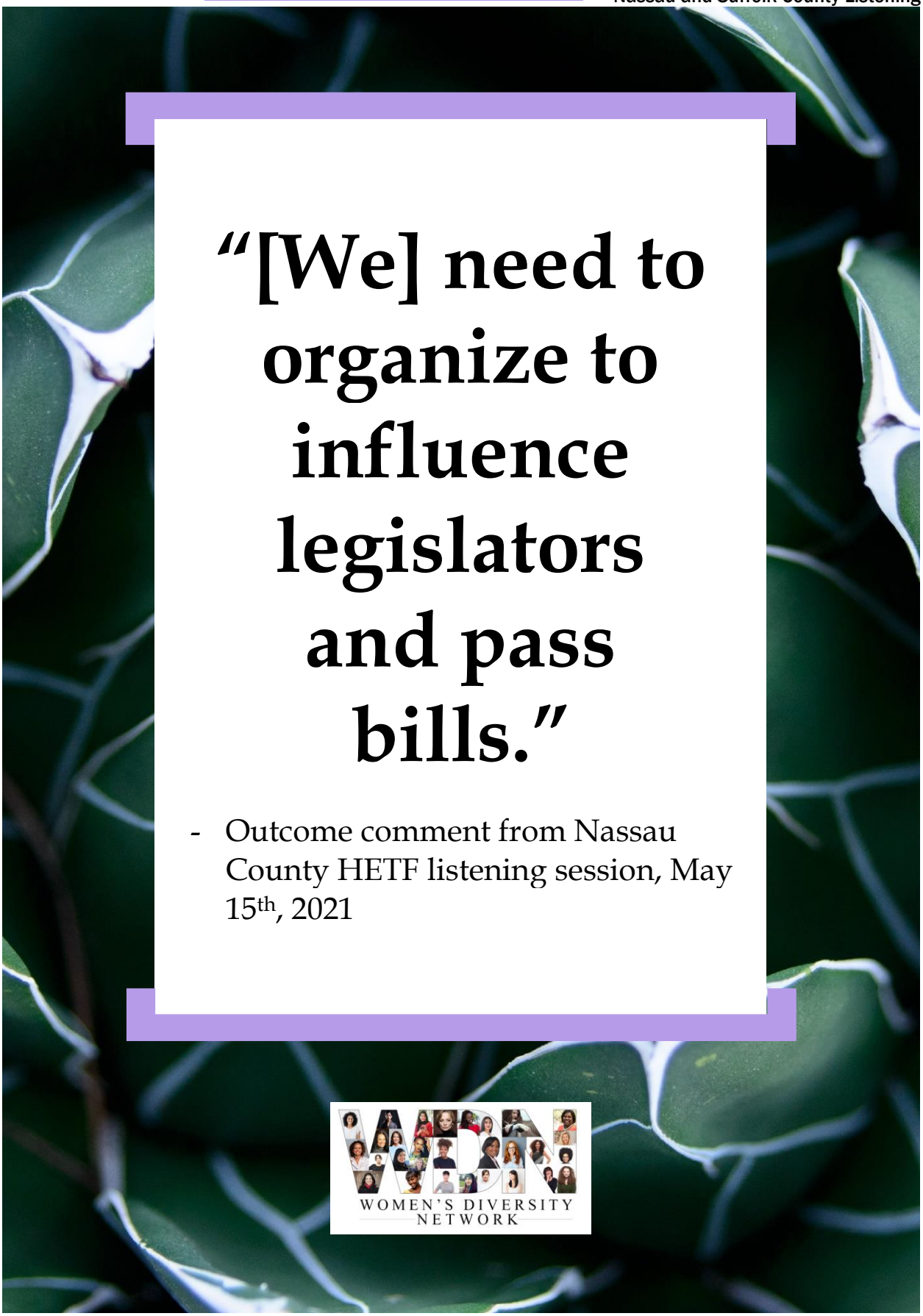
# Methods

Two HETF Virtual Maternal Mortality Forums were held in the spring of 2021: 1) Nassau County Virtual Maternal Mortality Forum, May 15th; and Suffolk County Virtual Maternal Mortality Forum, June 12th.




Participant responses were collected and organized by specific themes to determine recommendations in each of the identified areas of focus.


HETF Virtual Forum Goals
Develop strategies to reduce disparities in maternal health
Bring together local providers from various Long Island Health Systems
Identify best practices in perinatal care including preconception, prenatal, intranatal and postpartum.
Increase transparency and encourage accountability



# "[We] need to organize to influence legislators and pass bills."

- Outcome comment from Nassau County HETF listening session, May 15th, 2021



# Nassau County Listening Session

The issue of Black maternal health has grown more urgent in recent decades. A collective movement is needed to turn the tide of a trend that translates directly into adverse severe maternal morbidities. The process for building a public outcry around this issue is to 1) raise public awareness through continued discussion with healthcare practitioners, patient advocates, doulas, and potentially regulators; 2) write, propose, and pass legislation to protect the rights of pregnant and postpartum people; and, 3) expand access to services like doulas and translators through Medicaid. Public forums like this are only a first step towards building a public opinion that pushes lawmakers and healthcare systems to enforce accountability and ultimately structural change.




Doctors were reported in about half of the pregnancy-related deaths investigated in the 2014 NYS Maternal Mortality Review. While the identities of those reported were not made available to the public, there is also a need to share information about health practitioners between pregnant and postpartum people. The IRTH app is an online space to review and provide feedback on treatment and level of service provided by healthcare systems and health practitioners for public consumption. Furthermore, patient advocates and health practitioners need to listen to feedback and be held accountable when necessary.

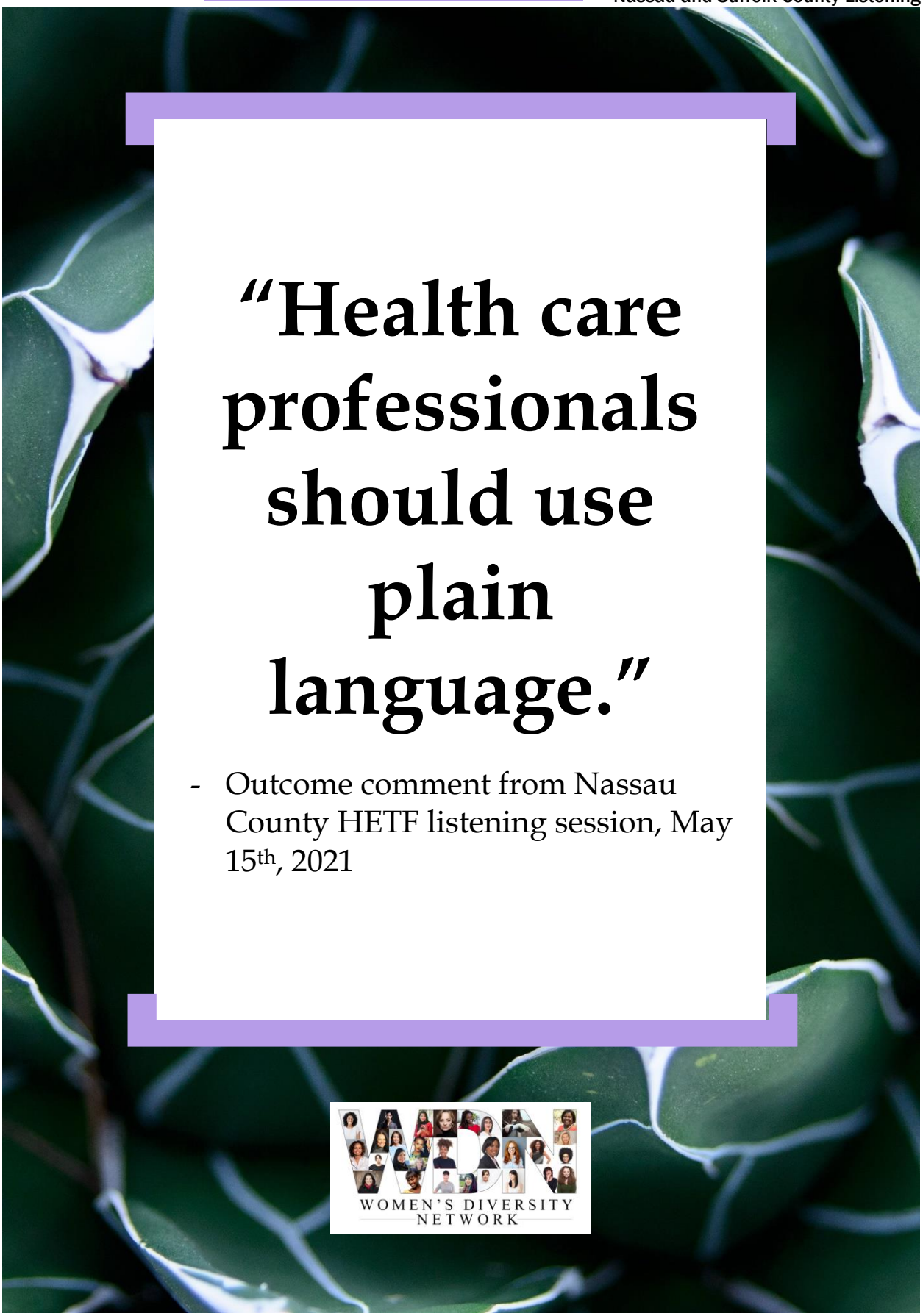


# "[Patients] need to have a good advocate to get the needed services."

- Outcome comment from Nassau County HETF listening session, May 15th, 2021




Doctors and healthcare systems can make changes to their policies that protect pregnant and postpartum patients. Thorough vetting of doctors and continued certification and education of providers can mitigate risk in the birthing process. Importantly, doctors and healthcare systems should survey patients and listen to patients' concerns. When a patient is unable to understand the language and terminology a doctor is using it disempowers them to make informed decisions about their own healthcare. By speaking in plain words and having a translator available this potentially can be avoided altogether.



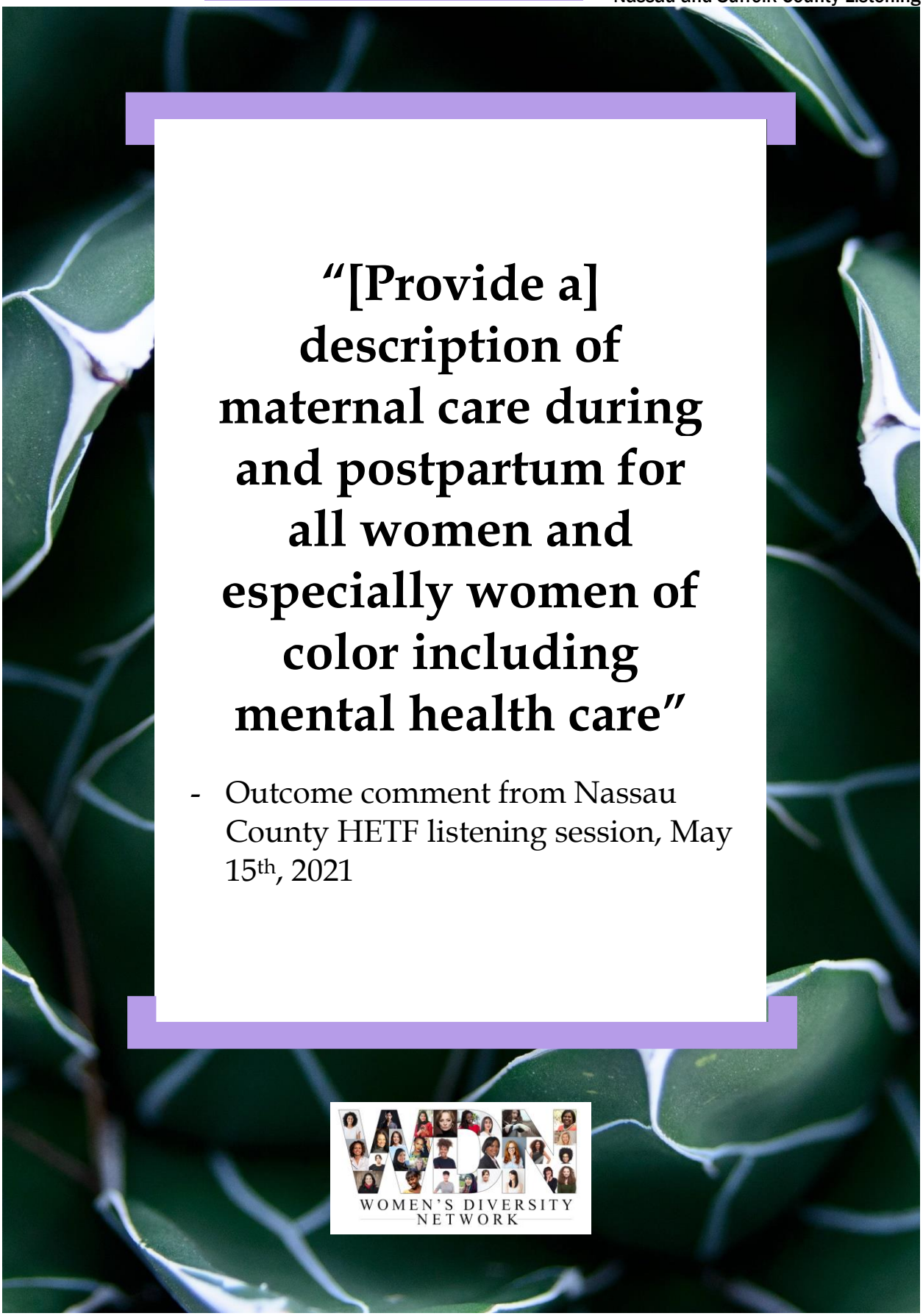
# "Health care professionals should use plain language."

- Outcome comment from Nassau County HETF listening session, May 15th, 2021




While doctors and healthcare practitioners can proactively respond to patient needs, patient education can prevent miscommunication. Medical language training and self-efficacy training can help patients advocate for themselves assertively.

The support structure for a patient can improve outcomes throughout a pregnancy and postpartum periods. Access to mental health services at these crucial times became a theme during the public forum.

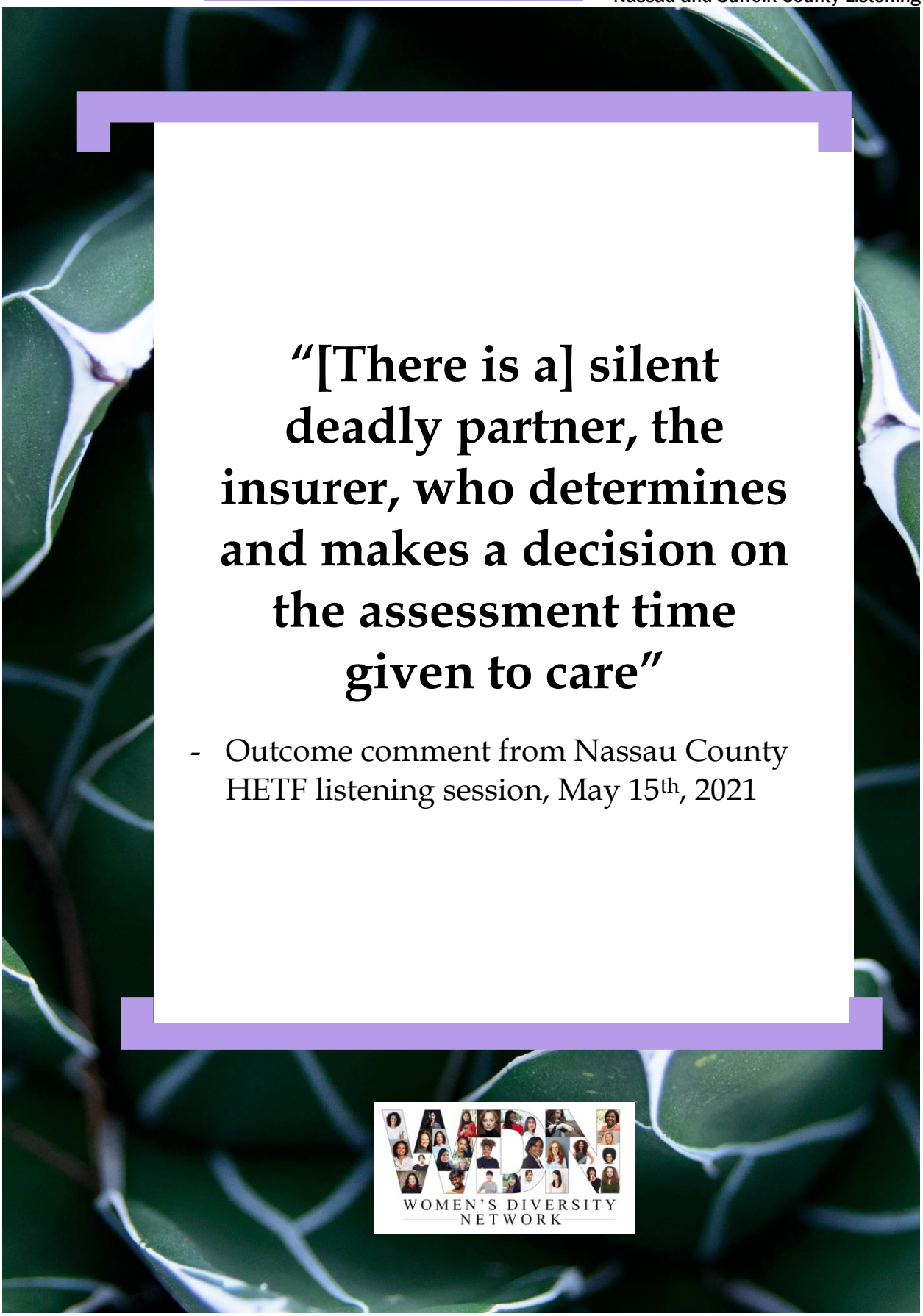


# "[Provide a] description of maternal care during and postpartum for all women and especially women of color including mental health care"

- Outcome comment from Nassau County HETF listening session, May 15th, 2021




It was widely acknowledged during the listening session that insurance can tie the hands of doctors by driving up the value of beds and preventing proper care and treatment, removing the ability of doctors to use their own best judgment. Privatization of hospitals can exacerbate this issue by introducing an inappropriate profit motive in healthcare.



**“[There is a] silent deadly partner, the insurer, who determines and makes a decision on the assessment time given to care”**

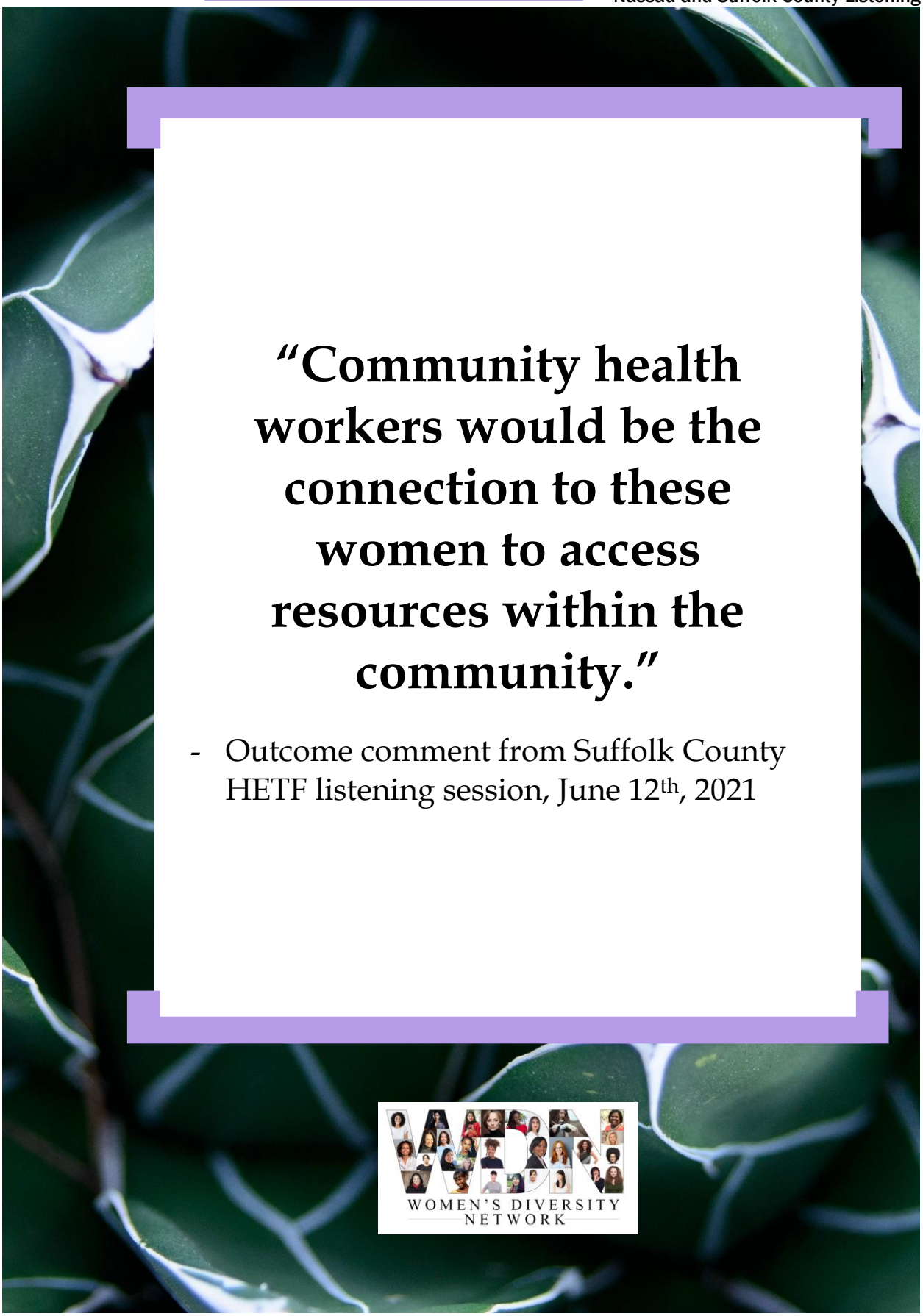
- Outcome comment from Nassau County HETF listening session, May 15th, 2021



This Photo by Unknown Author is licensed under CC BY-SA-NC

# Suffolk County Listening Session

Comments supporting healthcare provider accountability and resource sharing centered on community engagement. Friendly community organizations that work with grassroots community groups can empower women of color to access the support needed during and after pregnancy. Participants were focused on the role of community as a support network that includes health experts to provide education and resources. Doulas and community health workers are in high demand on Long Island, where there is a shortage of paid staff to guide pregnant and postpartum people through their healthcare experience. One proposed solution for this was an apprenticeship program for doulas. Such a program should focus on training staff of diverse race and ethnicities. One solution to engage diverse communities is to propose Science and Technology Entry Programs (STEP) with sponsorships available for children from low- and moderate-income households.



**"Community health workers would be the connection to these women to access resources within the community."**

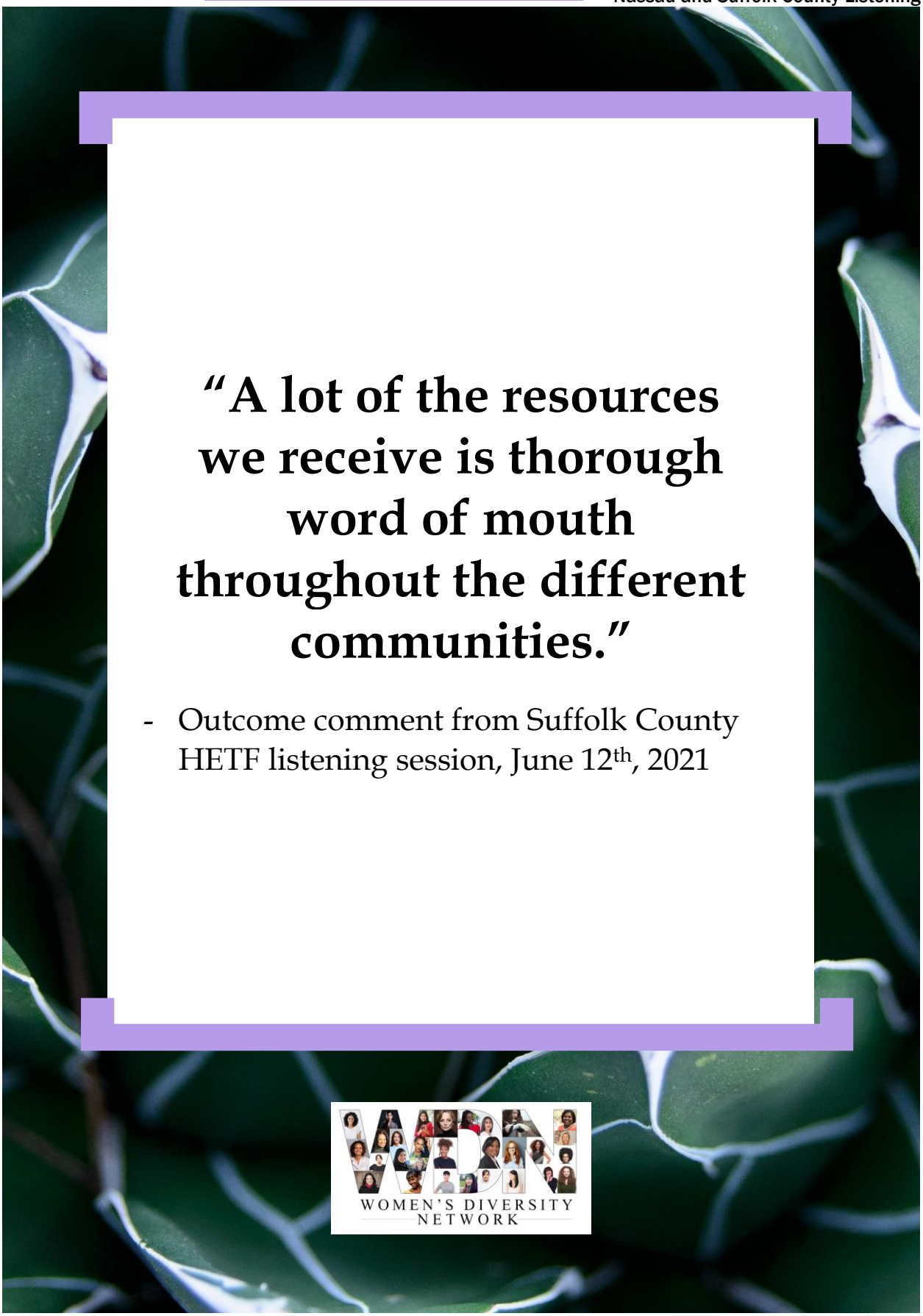
- Outcome comment from Suffolk County HETF listening session, June 12th, 2021



This Photo by Unknown Author is licensed under CC BY-SA

Many comments specifically addressed how and about which topics our membership can introduce the subject of maternal health outcomes with friends and family. The ability to distribute important information via word of mouth is the best way to create the communities of support we are seeking.

These comments ranged from encouraging people of color and Black people to enter the medical professions to communication style and building a community of trust within our own ranks. One topic of note was about how to address the difference between Black and non-Black women's pregnancy experiences in the context of maternal mortality. While maternal mortality is an issue facing all pregnant and postpartum people, Black maternal mortality is of particular urgency.



**"A lot of the resources we receive is thorough word of mouth throughout the different communities."**

- Outcome comment from Suffolk County HETF listening session, June 12th, 2021

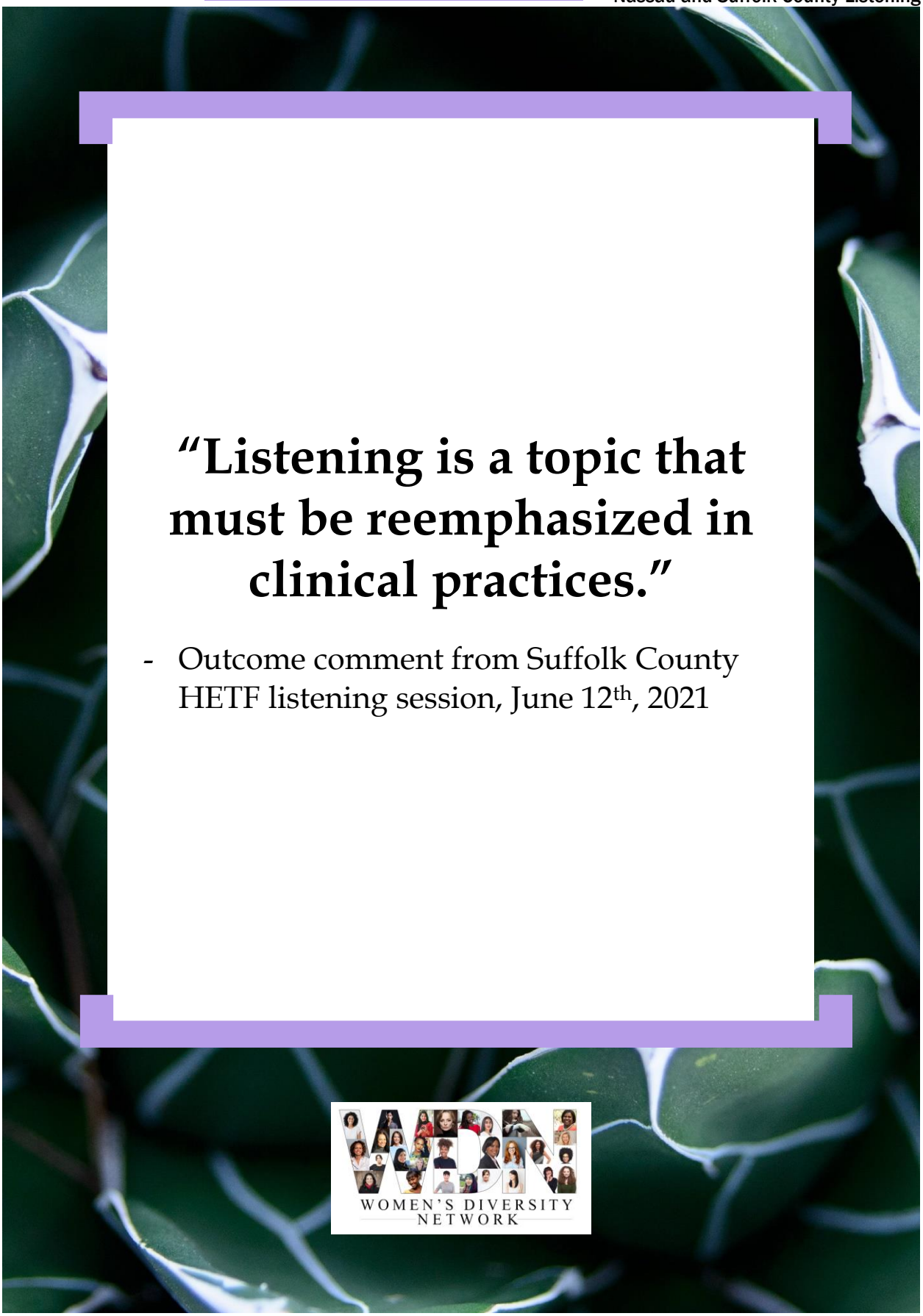


This Photo by Unknown Author is licensed under CC BY-NC-ND

On one hand, participants were concerned that all their providers would listen and treat patients with dignity and respect by 1) making sure that they are aware of their clients' medical histories and 2) understanding and being aware of implicit bias, 3) the potential for a client to have past experience with racism in their healthcare and 4) being aware of and respectful of cultural differences.

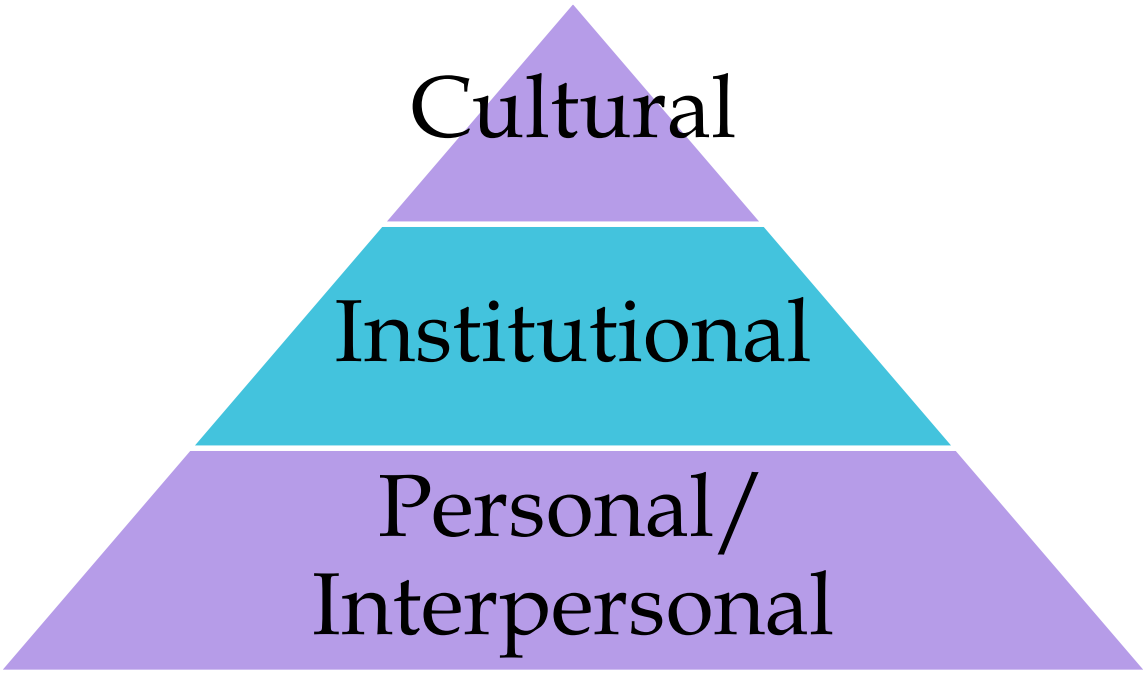
On the other hand, participants were also interested in the power of a team of caring professionals to make a childbirth experience successful. Everyone from the front desk to the delivering attendant to the 6-week checkup has a role to play in the process. A comprehensive continuity of care requires a team of healthcare workers to support pregnant and postpartum patients throughout the process. For instance, a pediatrician can engage new mothers and parents on resources and parenting skills training.

So, the vision of our participants is broad and holistic care from healthcare providers at all stages of development. However, insurance coverages are a barrier for patients to access perinatal services including in the 4th trimester and beyond. This includes coverage for lactation consultants and doulas.



# "Listening is a topic that must be reemphasized in clinical practices."

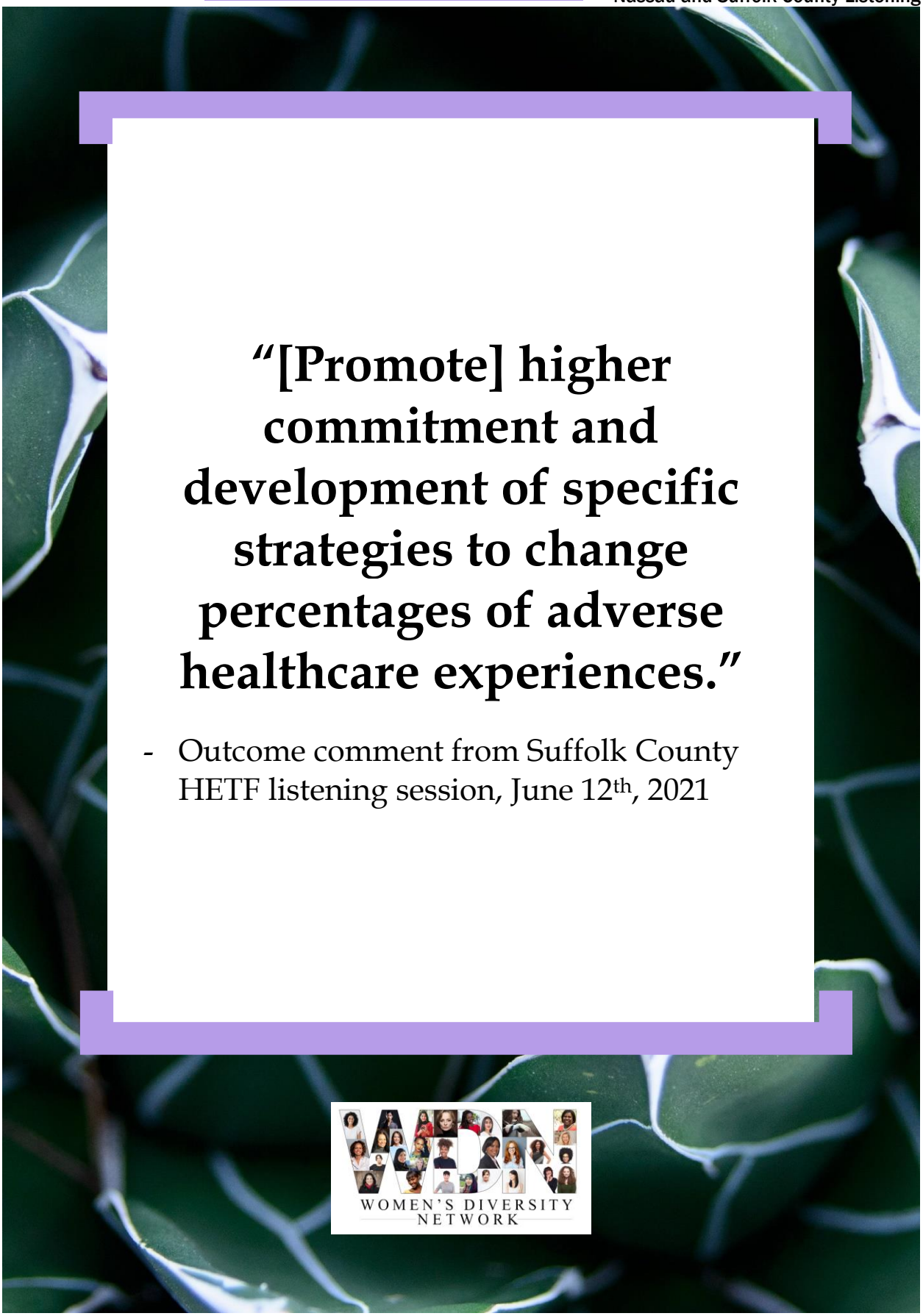
- Outcome comment from Suffolk County HETF listening session, June 12th, 2021




While interactions with healthcare providers and childbirth support team are the basis of many solutions to perinatal care, hospitals, health care systems and insurance companies can improve patient-provider interactions by creating a culture of accountability and patient-centered care.

Participants suggested that hospitals and other community organizations provide 1) a "birth box" with all the supplies needed for newborns in a box that the baby can even sleep in, and 2) postpartum parenting pamphlets with information about mental health and other resources for new mothers and parents.

Furthermore, administrators can encourage a doctor or social worker to screen new parents for common needs and refer their clients to appropriate resources. Further research should be conducted to determine specific strategies to reduce percentages of adverse healthcare experiences that includes hospitals and community stakeholders.



**"[Promote] higher commitment and development of specific strategies to change percentages of adverse healthcare experiences."**

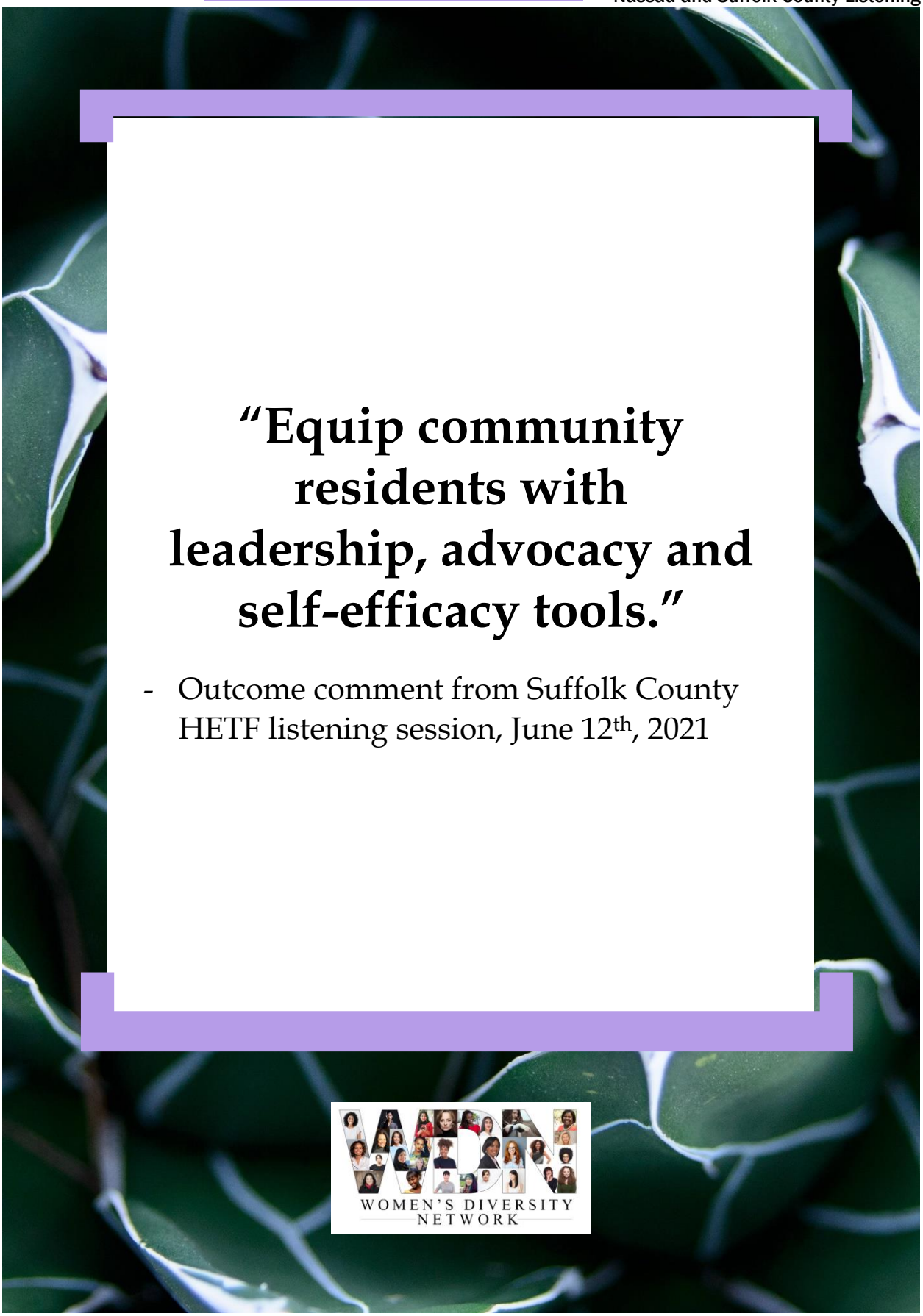
- Outcome comment from Suffolk County HETF listening session, June 12th, 2021



This Photo by Unknown Author is licensed under CC BY

Information sharing among communities can be empowering. Unfortunately, many patients are afraid to speak out about their treatment. There are many potential reasons for this including lack of understanding of medical terminology, lack of trust between provider and patient due to past experiences, like immigration status or medical racism, or even a language barrier.

Our participants are experts in their own experiences and as such have a great wealth of knowledge. While our participants actively listened to each other it became clear that peer-to-peer education is an important tool to break down barriers to the care pregnant and postpartum people need. How we listen to each other should reflect the active listening of a care team with their client.
It is important to participants to be aware of the influence of white supremacy acting within healthcare organizations and be prepared to report poorly delivered services and sharing that information with their communities.



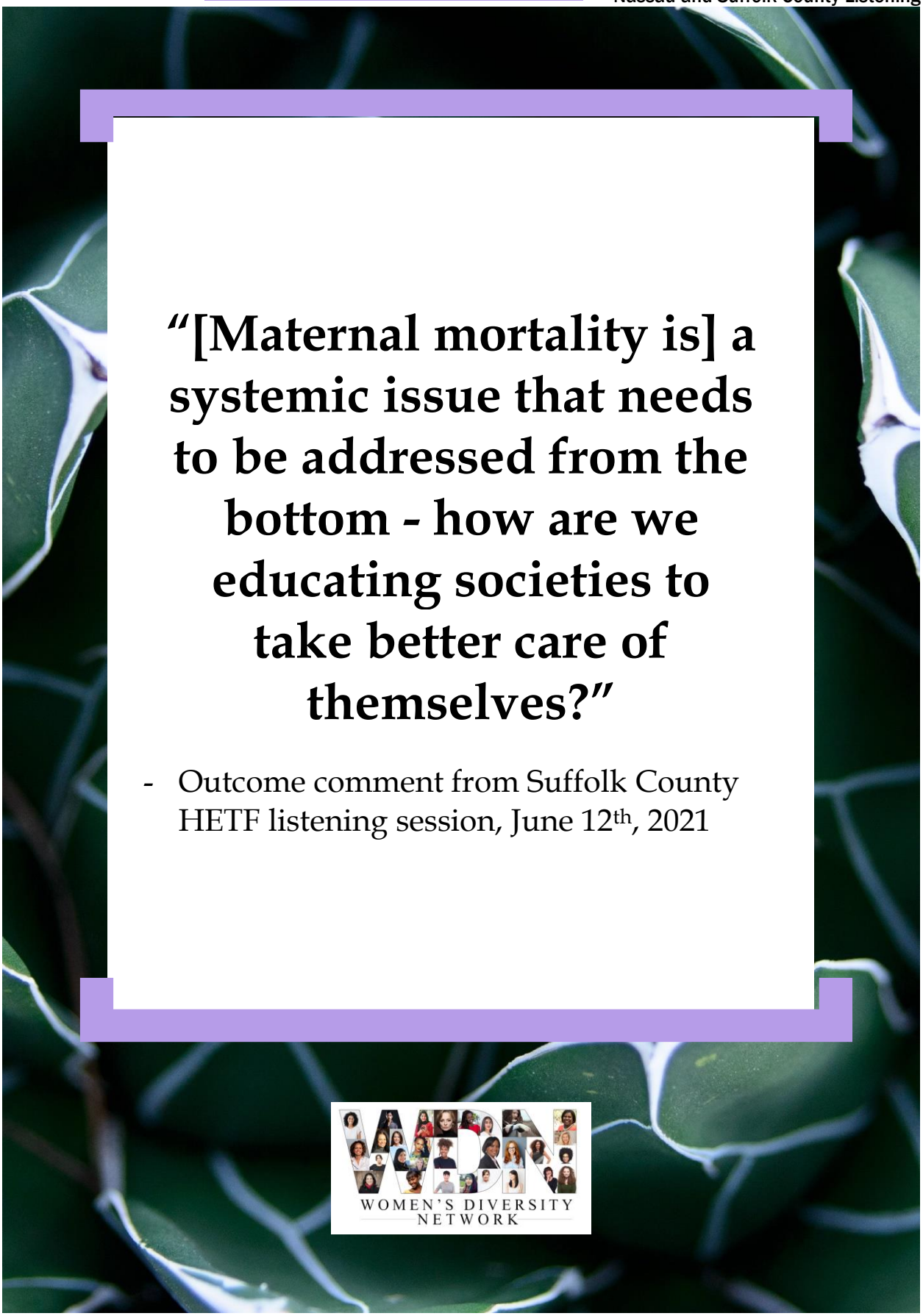
# "Equip community residents with leadership, advocacy and self-efficacy tools."

- Outcome comment from Suffolk County HETF listening session, June 12th, 2021



This Photo by Unknown Author is licensed under CC BY-SA

A powerful theme that came through in these comments is the shortcomings of society to center pregnancy and parenthood in our values. There remains a wide gap between the expectation of maternal and child health outcomes and reality. It is important to participants to acknowledge and respect the distance required to reach our goals. Our vision of childbirth includes a society in which all aspects of health are valued and centered.



**"[Maternal mortality is] a systemic issue that needs to be addressed from the bottom - how are we educating societies to take better care of themselves?"**

- Outcome comment from Suffolk County HETF listening session, June 12th, 2021




Word cloud produced from Suffolk County Listening Session participant responses
https://www.freewordcloudgenerator.com/generatewordcloud

An urgent problem such as maternal mortality requires collective action. While institutions and insurance companies fail to provide adequate childbirth services a regulation can raise awareness and enforce best practices. Participants were interested in legislative action at the local, state and federal level. There were a range of suggestions that primarily called for more access to resources for pregnant and postpartum people and accountability for healthcare providers.

Some program-based or technological solutions came up during our discussion. These were specific things that could be implemented but required funding or program development to achieve. These included tools to share information about healthcare experiences, accessibility of childbirth services, a doctor's checklist for perinatal visits, recruitment of community health workers on Long Island, and virtual perinatal visits.

# "How do we cultivate a network that reaches the people who don't have a network?"

- Outcome comment from Suffolk County HETF listening session, June 12th, 2021

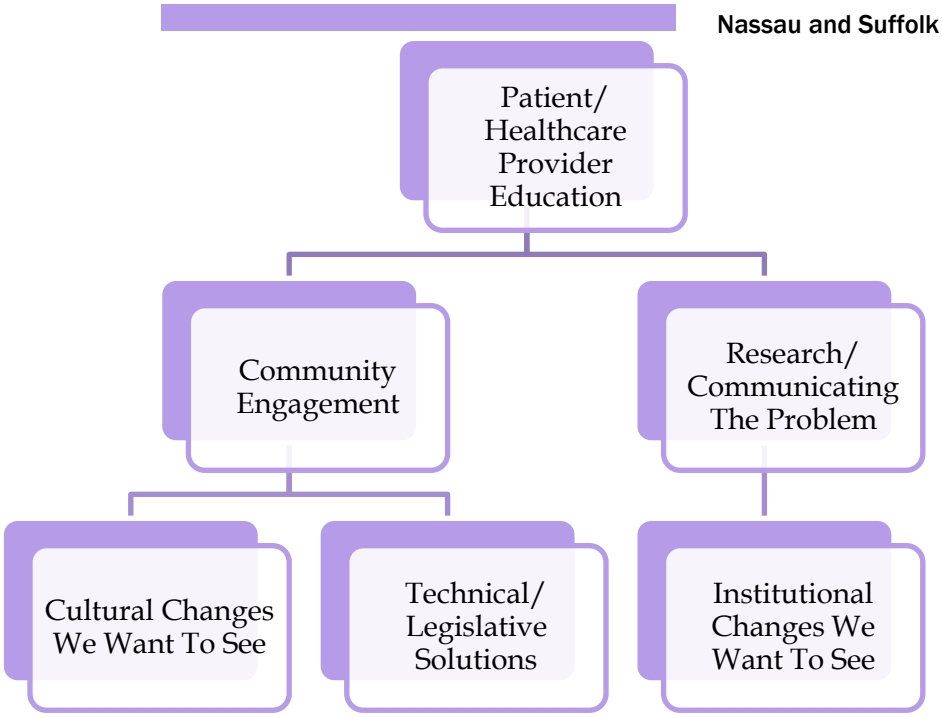
# Discussion

Themes from both listening sessions shed light on the experiences of pregnant and postpartum people from New York City and Long Island. Repeatedly, proper public health education was reported as a key need in the communities of our participants. Personal education about the social determinants of health came up again and again. All parties involved in healthcare should be well aware of the history of racism in public health and medicine. However, this forum reports that there is much work to be done in this sphere.

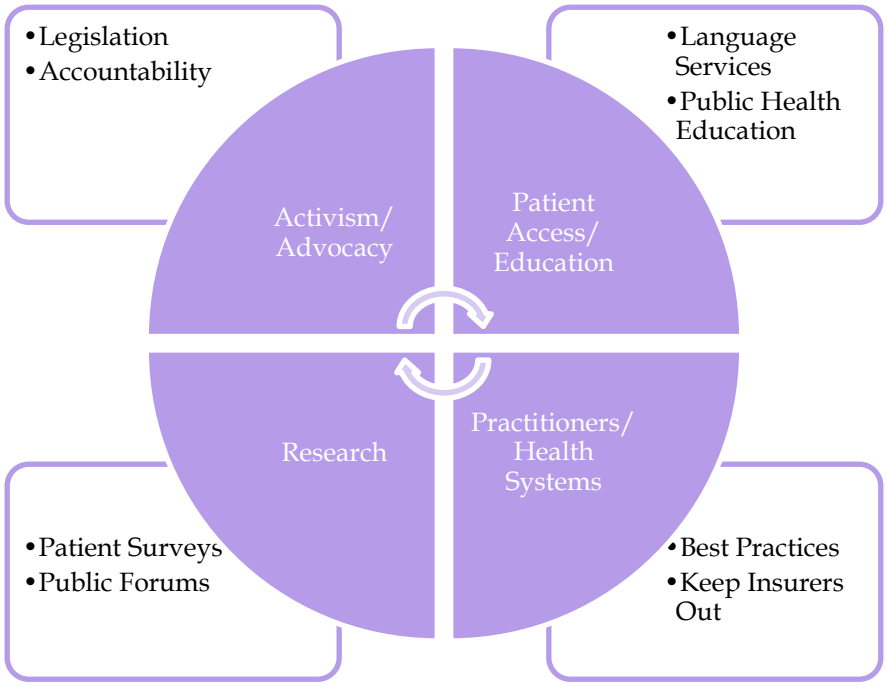
Another overarching theme is the ability of patients to self-advocate, be heard, and hold accountable healthcare providers who cannot or will not provide adequate services and referrals. A key feature of this call for better communication and transparency between patients and healthcare workers is language services. In particular, healthcare systems should recruit and prioritize bi-lingual workers at all levels.

Some solutions were practical things that could be done today if funded and implemented. Others were long-term visions of what healthcare should look like for pregnant and postpartum people - changes we want to see among medical school curricula, hospital and healthcare systems, insurance companies, and even at the societal level.

Yet, the most tangible solution to the Black maternal health crisis is in our hands. Activism and legislation are the most effective tools we have for making lasting change.




Suffolk County Themes Hierarchy Cascade




Nassau County Themes Cycle Matrix

# Conclusion

**"[We] Need A Collective Movement" -participant, May 15th, 2021**

There are many factors contributing to the maternal mortality crisis. In holding these listening sessions, HETF attempts to fill a gap in qualitative research surveys with women as study its primary subjects. Participants were asked to identify the needs for pregnant and postpartum people to have a successful birthing process.

By continuing to bring community members and healthcare practitioners together, the end goal for this project becomes clear. It behooves one to support the policies and procedures in the best interest of pregnant and postpartum people. This means that we need to understand the best practices for doctors and patients during perinatal care, including postpartum care.

An organized community can advocate for themselves. The first step to getting organized is to understand the solutions to our collective struggle for safe and healthy communities.